
UNIT 2 BEHAVIOURAL THERAPY, COGNITIVE BEHAVIOUR THERAPY AND APPROACHES TO COUNSELLING

Structure

- 2.0 Introduction
- 2.1 Objectives
- 2.2 Behaviour Therapy
 - 2.2.1 History
 - 2.2.2 Systematic Desensitisation
 - 2.2.3 Exposure: An extinction Approach
 - 2.2.4 Aversion Therapy
 - 2.2.5 Operant Conditioning Treatments
- 2.3 Cognitive Behavioural Therapy (CBT)
 - 2.3.1 Brief History
 - 2.3.2 ABC Model of CBT
 - 2.3.3 Goals of CBT
 - 2.3.4 Techniques/ Principles Used in CBT
 - 2.3.5 Levels of Cognition
 - 2.3.6 Techniques Used in CBT
 - 2.3.7 Hierarchical Structure of A-B-C
- 2.4 Let Us Sum Up
- 2.5 Unit End Questions
- 2.6 Suggested Readings

2.0 INTRODUCTION

This unit deals with behaviour therapy cognitive behaviour therapy and related issues. We start with Behaviour Therapy, how it started and with what purpose and proceed to give some of the techniques related to behaviour therapy. The techniques such as systematic desensitisation, exposure, flooding, and aversion therapy are all discussed in this unit. Then we take up operant conditioning techniques under which we discuss the positive reinforcement, participant modeling and assertiveness training etc. This is then followed by cognitive behaviour therapy in which we discuss its history, present the ABC model of CBT and describe the techniques of CBT in detail. We then present the hierarchical structure of ABC model of cognitive behaviour therapy and elucidate the principles underlying cognitive therapies.

2.1 OBJECTIVES

After completing this unit, you will be able to:

- Define behaviour therapy;
- Describe classical conditioning procedures in behaviour therapy;

- Explain cognitive-behavioural therapy (CBT) and its principles;
- Describe A-B-C Model of CBT;
- Identify the difference between behaviour therapy and cognitive-behavioural therapy (CBT); and
- Explain the techniques involved in CBT.

2.2 BEHAVIOUR THERAPY

Behavioural psychology, or behaviourism, arose in the early 20th century in reaction to the method of introspection that dominated psychology at the time. John B. Watson, the father of behaviourism, had initially studied animal psychology. In the 1960s, behaviour approaches emerged as a dramatic departure from the assumptions and methods that characterised psychoanalytic and humanistic therapies. They argued that psychology should concern itself only with publicly observable phenomena i.e., overt behaviour. According to Behaviouristic thinking, as mental content is not publicly observable, thus it cannot be subjected to rigorous scientific inquiry.

The new practitioners of behaviour therapy denied the importance of inner dynamics, instead they insisted that

- 1) maladaptive behaviours are not merely symptoms of underlying problems but rather are problems;
- 2) problem behaviours are learned on the same ways normal behaviours are; and
- 3) maladaptive behaviours can be unlearned by applying principles derived from research on classical conditioning, operant conditioning and modeling.

Consequently, behaviourists developed a focus on overt behaviours and their environmental influences.

Behaviour therapy involves changing the behaviour of clients to reduce dysfunction and to improve quality of life. Behaviour therapy includes a methodology, referred to as *behaviour analysis*, for the strategic selection of behaviours to change, and a technology to bring about behaviour change, such as modifying antecedents or consequences or giving instructions. Behaviour therapy represents clinical applications of the principles developed in learning theory.

2.2.1 History

Around 1920s, the application of learning principles to the treatment of behavioural disorders began to appear, but it had little effect on the mainstream of psychiatry and clinical psychology. Behaviour therapy emerged as a systematic and comprehensive approach to psychiatric (behavioural) disorders in 1960s. Joseph Wolpe and his colleagues used pavlovian techniques to produce and eliminate neuroses in cats. From this research, Wolpe developed systematic desensitisation. At about the same time, Eysenck and Shapiro stressed the importance of an experimental approach in understanding and treating individual patients, using modern learning theory. A Harvard psychologist B. F. Skinner also inspired the origin of behaviour therapy. Skinner's students began to apply his operant conditioning technology, developed in animal conditioning laboratories, to human beings in clinical settings.

Classical conditioning procedures have been used in two major ways.

- i) First, they have been used to reduce or de condition anxiety responses.
- ii) Second, they have been used in attempts to condition aversive stimuli.

His most commonly used classical conditioning procedures are:

- Systematic Desensitisation,
- Exposure therapy, and
- Aversion therapy.

Let us take up each of these and discuss.

2.2.2 Systematic Desensitisation

In 1958 Joseph Wolpe introduced Systematic Desensitisation, This is actually learning based treatment for anxiety disorders. Wolpe viewed anxiety as a classical conditioned response. His goal was to eliminate anxiety by using a procedure called Counterconditioning. In this, a new response that is incompatible with anxiety is produced. For example relaxed state which is incompatible with anxiety state. This relaxed state is conditioned to the anxiety arousing conditioned stimulus (CS), like for instance fear of a closed room or fear of heights etc.

It is based on the behavioural principle of Counterconditioning, whereby a person overcomes maladaptive anxiety elicited by a situation or an object by approaching the feared situation gradually, in a psychophysiological state that inhibits anxiety.

In systematic desensitisation, patients attain a state of complete relaxation and are then exposed to the stimulus that elicits the anxiety response.

The negative reaction of anxiety is inhibited by the relaxed state, a process called *reciprocal inhibition*.

Instead of using actual situations or objects that elicit fear, patients and therapists prepare a graded list or hierarchy of anxiety provoking scenes associated with a patient's fears. The learned relaxation state and the anxiety provoking scenes are systematically paired in treatment. Thus, systematic desensitisation consists of three steps:

- Hierarchy construction
- Relaxation training
- Desensitisation of the fear evoking stimulus.

Step 1: Hierarchy Construction

When constructing a hierarchy, clinicians determine all the conditions that elicit anxiety, and then patients create a hierarchy list of 10 to 12 scenes that produce anxiety and these are then put in order of increasing anxiety. In the box below is given an example of a hierarchy construction used in the systematic desensitisation treatment of a Test Anxious College Student.

Scene: Hierarchy of Anxiety Arousing Scenes

Hearing about someone else who has a test (**Least anxiety provoking**)
Instructor announcing that a test will be given in three weeks
Instructor reminding class that there will be a test in two weeks
Overhearing classmates talk about studying for the test, which will occur in one week
Instructor reminding class of what it will be tested on in two days
Leaving class the day before exam
Studying the night before test
Getting up morning of the test
Walking toward the building where the exam will be given
Walking into the exam room
Instructor walking into the room with the test
Tests being distributed
Reading the test questions
Watching others finishing the test
Seeing a question I can not answer
Instructor waiting for me to finish the test (**High anxiety provoking**)

Step 2: Relaxation Training

Relaxation training can help people control their physiological responses in stressful situations. These techniques produce physiological effects opposite to those of anxiety, that is slow heart rate, increased peripheral blood flow, and neuromuscular stability.

A variety of relaxation methods have been developed. Most methods use progressive relaxation, also known as somatic relaxation training, developed by the psychiatrist Edmund Jacobson.

It provides a means of voluntarily reducing or preventing high level of arousal. Patients relax major muscle groups in a fixed order, beginning with the small muscle groups of the feet and working towards shoulders, neck, face and head etc. Moving from lowest extremities to the head is called cephalocaudal. Ofcourse one can also start with head muscles, face muscles and move down to neck etc. and then on to the extremities.

Some clinicians use hypnosis to facilitate relaxation or use tape-recorded exercise to allow patients to practice relaxation on their own.

Mental imagery is a relaxation method in which patients are instructed to imagine themselves in a place associated with pleasant relaxed memories.

Such images allow patients to enter a relaxed state or experience. The physiological changes that take place during relaxation are the opposite of those induced by the stress responses that are part of many emotions.

Muscle tension, respiration rate, heart rate, blood pressure, and skin conductance decrease. Relaxation increases respiratory heart rate variability, which is an index of parasympathetic tone. In the box below relaxation step by step are given

With its focus on full, cleansing breaths, deep breathing is a simple, yet powerful, relaxation technique. It's easy to learn, can be practiced almost anywhere, and provides a quick way to get your stress levels in check. Deep breathing is the cornerstone of many other relaxation practices, too, and can be combined with other relaxing elements such as aromatherapy and music. All you really need is a few minutes and a place to stretch out.

How to practice deep breathing

The key to deep breathing is to breathe deeply from the abdomen, getting as much fresh air as possible in your lungs. When you take deep breaths from the abdomen, rather than shallow breaths from your upper chest, you inhale more oxygen. The more oxygen you get, the less tense, short of breath, and anxious you feel. So the next time you feel stressed, take a minute to slow down and breathe deeply:

- Sit comfortably with your back straight. Put one hand on your chest and the other on your stomach.
- Breathe in through your nose. The hand on your stomach should rise. The hand on your chest should move very little.
- Exhale through your mouth, pushing out as much air as you can while contracting your abdominal muscles. The hand on your stomach should move in as you exhale, but your other hand should move very little.
- Continue to breathe in through your nose and out through your mouth. Try to inhale enough so that your lower abdomen rises and falls. Count slowly as you exhale.

If you have a hard time breathing from your abdomen while sitting up, try lying on the floor. Put a small book on your stomach, and try to breathe so that the book rises as you inhale and falls as you exhale.

Progressive muscle relaxation for stress relief

Progressive muscle relaxation is another effective and widely used strategy for stress relief. It involves a two-step process in which you systematically tense and relax different muscle groups in the body.

With regular practice, progressive muscle relaxation gives you an intimate familiarity with what tension—as well as complete relaxation—feels like in different parts of the body. This awareness helps you spot and counteract the first signs of the muscular tension that accompanies stress. And as your body relaxes, so will your mind. You can combine deep breathing with progressive muscle relaxation for an additional level of relief from stress.

Progressive Muscle Relaxation Sequence

- Right foot
- Left foot
- Right calf
- Left calf
- Right thigh

- Left thigh
- Hips and buttocks
- Stomach
- Chest
- Back
- Right arm and hand
- Left arm and hand
- Neck and shoulders
- Face

Most progressive muscle relaxation practitioners start at the feet and work their way up to the face. For a sequence of muscle groups to follow, see the box to the right:

- Loosen your clothing, take off your shoes, and get comfortable.
- Take a few minutes to relax, breathing in and out in slow, deep breaths.
- When you're relaxed and ready to start, shift your attention to your right foot. Take a moment to focus on the way it feels.
- Slowly tense the muscles in your right foot, squeezing as tightly as you can. Hold for a count of 10.
- Relax your right foot. Focus on the tension flowing away and the way your foot feels as it becomes limp and loose.
- Stay in this relaxed state for a moment, breathing deeply and slowly.
- When you're ready, shift your attention to your left foot. Follow the same sequence of muscle tension and release.
- Move slowly up through your body — legs, abdomen, back, neck, face — contracting and relaxing the muscle groups as you go.

Step 3: Desensitisation of the Anxiety Provoking Stimulus

In the final step, called *desensitisation*, the patient proceeds systematically through the list from the least, to the most, anxiety provoking scene while in a deeply relaxed state. In fact they vividly imagine the situation as is given in the list and as they imagine and as they are in the relaxed state, the scene even if evokes anxiety the anxiety does not bother the individual. In case it does, then the patient is made to relax once again all the more and the same scene is imagined, until the anxiety is no more evoked when visualising the concerned anxiety provoking situation. Once the patient is totally relaxed even under the anxiety provoking situation, the patient is asked to move to the next item in the list that is anxiety provoking. The same process as above is repeated until the situation no more evokes anxiety. Then the patient moves on to the third item in the list and so on. The rate at which the clients progress through the list is determined by their responses to the stimuli. When patients can vividly imagine the most anxiety provoking scene of the hierarchy with equanimity, they experience little anxiety in the corresponding real life situation.

2.2.3 Exposure: An Extinction Approach

From a behavioural point of view, phobias and other fears result from classically conditioned emotional responses. The conditioning experience is assumed to involve a pairing of the phobic object (fear of heights let us say) with an aversive unconditioned stimulus (UCS). For example, if a person has a fear of heights, then put him at a height (UCS) where he has no way to come down except with the help of a ladder that you may provide him later. As a result, the phobic stimulus (Height) becomes a conditioned stimulus (CS) that elicits the conditioned response (CR) of anxiety. Continuously staying exposed to the aversive stimulus (heights) in course of time brings down the anxiety and the extreme negative reaction. As this process of exposure is repeated again and again, the patient does not react negatively or with fear when exposed to heights.

Thus, according to this formulation, the most direct way to reduce the fear is through a process of classical extinction of the anxiety response. The client may be exposed to real life stimuli or asked to imagine scenes involving the stimuli. These stimuli will evoke considerable anxiety, but the anxiety will be extinguished in time if the person remains in the presence of the fear evoking stimulus (heights) and the UCS (anxiety) does not occur.

Some critics of exposure treatment are concerned that the intense anxiety evoked by the treatment may worsen the problem or cause clients to flee from the treatment. But experimental research has proved this technique a very effective treatment for extinguishing anxiety in both animals and humans.

Advances in computer technology have made it possible to present environmental cues in Virtual Reality (VR) for exposure treatment. Virtual reality involves the use of computer technology to create highly realistic virtual environments that stimulate virtual actual experience so vividly that they evoke many of the same reactions that a comparable real world environment would. Beneficial effects have been reported with virtual reality exposure of patients with height phobia, fear of flying, spider phobia, and claustrophobia. Much experimental work is being done in the field.

2.2.4 Aversion Therapy

For some clients, the therapeutic goal is not to reduce anxiety but actually to condition anxiety to a particular stimulus that triggers deviant behaviour. In aversion therapy, the therapist pairs a stimulus (drinking alcohol) that is attractive to the client (the CS) with a noxious UCS (vomiting due to an injection given immediately before taking alcohol) in an attempt to condition an aversion to the CS (alcohol). When a noxious stimulus (vomiting as punishment) is presented immediately after a specific behavioural response, theoretically, the response is eventually inhibited and extinguished. Many types of noxious stimuli are used which include for instance, electric shocks, substances that induce vomiting, corporal punishment, and social disapproval. The negative stimulus is paired with the behaviour, which is thereby suppressed. The unwanted behaviour may disappear after a series of such sequences. Aversion therapy has been used for alcohol abuse, paraphilias (child molesters), and other behaviours with impulsive or compulsive qualities, but this therapy is controversial for many reasons. For example, punishment does not always lead to the expected decreased response and can sometimes be positively reinforcing.

2.2.5 Operant Conditioning Treatments

The term behaviour modification refers to treatment techniques that apply operant conditioning procedures in an attempt to increase or decrease a specific behaviour like positive reinforcement, extinction, negative reinforcement, or punishment.

- i) **Positive Reinforcement:** When a behavioural response is followed by a generally rewarding event, such as food, avoidance of pain, or praise, it tends to be strengthened and to occur more frequently than before the reward. This principle has been applied in a variety of situations. In the inpatient hospital wards, patients with mental disorder receive a reward for performing a desired behaviour, such as tokens that they can use to purchase luxury items or certain privileges. The process, known as token economy, has successfully altered undesirable behaviours to desirable behaviours.
- ii) **Flooding:** Flooding, sometimes called *implosion*, is similar to graded exposure in that it involves exposing the patient to the feared object in vivo; however, there is no hierarchy. It is based on the premise that escaping from an anxiety provoking experience reinforces the anxiety through conditioning. Thus, clinicians can extinguish the anxiety and prevent the conditioned avoidance behaviour by not allowing patients to escape the situation.

In a variant, called *imaginal flooding*, the feared object or situation is confronted only in the imagination, not in real life.

Many patients refuse flooding because of the psychological discomfort involved.

It is also contraindicated when intense anxiety would be hazardous to a patient (e.g., those with heart disease or fragile psychological adaptation). The technique works best with specific phobias.

- iii) **Participant Modeling:** In participant modeling, patients learn a new behaviour by imitation, primarily by observation, without having to perform the behaviour until they feel ready. Learning by watching others' behaviour is also called Observational learning.

Just as irrational fears can be acquired by learning, they can be unlearned by observing a fearless model confront the feared object. The technique has been useful with phobic children who are placed with other children of their own age and sex who approach the feared object or situation.

With adults, a therapist may describe the feared activity in a calm manner that a patient can identify.

Or, the therapist may act out the process of mastering the feared activity with a patient.

Sometimes, a hierarchy of activities is established, with the least anxiety provoking activity being dealt with first. The participant modeling technique has been used successfully with agoraphobia by having a therapist accompany a patient into the feared situation. In a variant of the procedure, called *behaviour rehearsal*, real life problems are acted out under a therapist's observation or direction.

- iv) **Assertiveness Training:** Assertive behaviour enables a person to act in his or her own best interest, to stand up for herself or himself without undue anxiety, to express honest feelings comfortably, and to exercise personal rights without denying the rights of others.

Two types of situations frequently call for assertive behaviours:

- setting limits on pushy friends or relatives and
- commercial situations, such as countering a sales pitch or being persistent when returning defective merchandise.

Early assertiveness training programs tended to define specific behaviours as assertive or nonassertive.

- v) **Social Skills Training:** Social skills training have been found to be efficacious for depression and schizophrenics. These training programs cover skills in the following areas:

- conversation,
- conflict management,
- assertiveness,
- community living,
- friendship and dating,
- work and vocation, and
- medication management.

Each of these skills has several components. For example, assertiveness skills include the following:

- making requests,
- refusing requests,
- making complaints,
- responding to complaints,
- expressing unpleasant feelings,
- asking for information,
- making apologies,
- expressing fear, and
- refusing alcohol and street drugs.

Each component involves specific steps.

2.3 COGNITIVE BEHAVIOURAL THERAPY (CBT)

In 1960s, Cognitive revolution in Psychology occurred and by 1970 many behaviour therapists influenced by it began to call it Cognitive Behavioural therapy. Cognitive behavioural therapy (CBT) is an amalgam of two therapies: cognitive therapy (CT) and behavioural therapy. The term “cognitive behavioural therapy” is a very general term for a classification of therapies with similarities.

2.3.1 Brief History

CBT is a form of psychotherapy that emphasises the important role of thinking in how we feel and what we do. It does not exist as a distinct therapeutic technique. Beck and Ellis are two names associated with Cognitive Behavioural Therapy.

Albert Ellis's system, originated in the early 1950s, developed Rational Emotive Therapy (RET), the first discrete therapeutic approach to CBT. Aaron T. Beck, inspired by Albert Ellis, developed **cognitive therapy** in the 1960s.

Ellis developed and popularised the ABC model of emotions, and later modified the model to the A-B-C-D-E approach.

In the 1990's Ellis renamed his approach Rational Emotive Behaviour Therapy. Behavioural therapy focuses on a person's actions and aims to change unhealthy behaviour patterns.

In the 1960's, Aaron Beck, M.D. developed his approach called Cognitive Therapy. CT focuses on a person's thoughts and beliefs, and how they influence a person's mood and actions, and aims to change a person's thinking to be more adaptive and healthy. Aaron Beck proposed the basic principles of CBT which can be applied to variety of psychiatric and medical problems. It focuses on identifying and changing maladaptive patterns of information processing and related behaviours. CBT methods are rooted in a psychological understanding of symptoms. It shares three fundamental propositions:

- 1) Cognitive activity affects behaviour
- 2) Cognitive activity may be altered and monitored
- 3) Desired behaviour change may be effective through cognitive change

Beck's approach became *known for its effective* treatment of depression.

Cognitive behavioural approach is based on the assumption that anxiety, depression, and negative emotions develop from maladaptive thought processes.

2.3.2 ABC Model of CBT

This model records a sequence of events in terms of:

- A – Activating Event (described as a 'Trigger')
- B – Beliefs (e.g. the thoughts that occur when the Activating Event happens)
- C – Consequences (how you feel and behave when you have those Beliefs)

This ABC technique of irrational belief has proven as the major tool in cognitive therapy. The main aim of this technique is to focus around the causes that develop irrational belief and has to be recorded during the therapy.

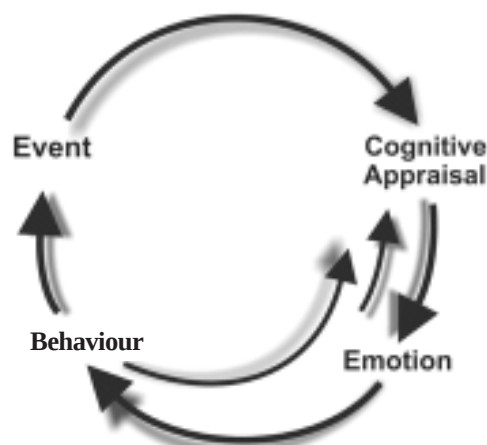


Fig.1: Basic Cognitive Behaviour Model

Source: From Wright JH. Basco MR. Thase ME: Learning Cognitive-Behaviour Therapy: An Illustrated Guide. Washington, DC. American Psychiatric Publishing. 2006, p 5

CBT helps a person focus on his or her current problems and how to solve them. Both patient and therapist need to be actively involved in this process. The therapist helps the patient learn how to identify distorted or unhelpful thinking patterns, recognise and change inaccurate beliefs, relate to others in more positive ways, and change behaviours accordingly. There are several approaches to cognitive behavioural therapy, including the following:

- 1) Rational Emotive Behaviour Therapy,
- 2) Rational Behaviour Therapy,
- 3) Rational Living Therapy,
- 4) Cognitive Therapy, and
- 5) Dialectic Behaviour Therapy.

2.3.3 Goals of CBT

CBT focuses on A *Here and Now* orientation.

The goals of CBT are:

- To challenge the thoughts about a particular situation by identifying the cognitive traps
- To help the patient to identify less threatening alternative and
- To test out these alternatives in the real world.
- It also aims to leave clients at the completion of therapy with the freedom to choose their emotions, behaviours and lifestyle (within physical, social and economic restraints).
- To Finally effect profound and lasting change in the underlying belief system of the client, rather than simply eliminating the presenting symptoms.
- To make the client equipped with self help techniques that enable coping in the longterm future

2.3.4 Techniques/Principles of CBT

CBT is based on several principles. These are:

- It is brief and time-limited
- It is structured, directive, and problem-oriented
- It is based on the cognitive-behavioural model of emotional disorders.
- It is a collaborative effort between the CBT practitioner and the client seeking treatment and requires a sound therapeutic relationship. They work together to try to understand the person's difficulties and what may be contributing to them.
- The sessions of CBT are typically one hour in length.
- The clients are guided to discover new ways of thinking for themselves with specific questions.
- It is often based on an education model.

- It relies on the inductive method, a scientific approach using logic and reasoning.
- It uses between-session practice as a central feature (for people to put into practice what they have learned). New behaviours are initially tested in safe situations (for example, the practitioner's office).
- It focuses on the present. The past cannot be changed, but the way we think about the past can be changed

2.3.5 Levels of Cognition

Three major levels of cognition, relevant to the practice of CBT, were identified by Beck and his colleagues. These are (i) Full consciousness, (ii) Automatic Thoughts, and (iii) Schemas.

- Consciousness is a state in which rational decisions are made with full awareness.
- Automatic thoughts are more autonomous, often private cognitions that flow rapidly in the stream of everyday thinking and may not be carefully assessed for accuracy or relevance.
- Schemas, the deepest level of cognition, are fundamental rules for information processing which are shaped by developmental influences and other life experiences. Schemas that play a major role in regulating self worth and behavioural coping strategies are a frequent target of CBT.

Examples of automatic thoughts and maladaptive schemas

Automatic Thoughts	Maladaptive schemas
I should be doing better in life	I must be perfect to be accepted
I have let him / her down	I am a fake
I always keep messing things up	If I choose to do something, I must succeed.
I cannot handle it	I am unlovable
It is too much for me	No matter what I do, I won't succeed
I don't have much of a future	The world is too frightening for me
Things are out of control	Others cannot be trusted
I feel like giving up	I must always be in control
I will never be able to get this done	I am stupid
Something bad is sure to happen	Other people will take advantage of me.

(Source: Wright et al (2011))

2.3.6 Techniques Used in CBT

The CBT techniques, described below, are useful in eradicating certain problems in a time bound manner which stem from maladaptive thoughts and behaviours. The techniques are given in the following section:

- i) **Cognitive Rehearsal:** In this technique, the therapist asks the patient to recall a problematic situation of the past. The therapist and patient both work together to find out the solution to the problem.
- ii) **Validity Testing:** This is one of the first cognitive therapy techniques. Every disorder or disease must have supporting evidence and the therapist asks the patient, to defend his feelings, thoughts and beliefs, with objective evidence. The faulty nature or invalidity of the beliefs of the patient is exposed if he is unable to produce any kind of objective evidence.
- iii) **Writing in a diary / Journal:** It is the practice of maintaining a diary to keep an account of the situations that arise in day to day life. This is a detailed diary, which notes every emotion, thought and belief. The therapist and the patient, later, collectively assess the diary/ journal, to analyse how they actually affect the behaviour of the individual..
- iv) **Guided Discovery:** The purpose of this technique is to enable the patient realise his cognitive distortions. The patient becomes aware of his cognitive distortions through a series of questions and answers.
- v) **Systematic Positive Reinforcement:** Cognition can be changed over a period of time, by positive encouragements and small rewards. It is one of the cognitive behavioural therapy techniques in which positive behaviours of a person are rewarded with a positive reinforcement.

A reward system is established for the reinforcement of certain positive behaviours. Just like positive reinforcement, withholding the reinforcement is also equally important. It helps in eradicating a maladaptive behaviour.

- vi) **Aversive Conditioning:** This technique makes use of dissuasion for lessening the appeal of a maladaptive behaviour. The patient is exposed to an unpleasant stimulus while engaging him in a particular behaviour or thought for which he has to be treated, thus, the unpleasant stimulus gets associated with such thoughts/behaviours and then the patient exhibits an aversive behaviour towards them.
- vii) **Modeling:** In Modeling, the patient makes use of the behaviour of the therapist as a model in order to solve his/her problems. The therapist performs role playing exercises which are aimed at responding in an appropriate way to overcome difficult situations.
- viii) **Homework:** The therapist gives a set of assignments to patients as homework. The patient take notes during the session, reviews the audiotapes of a particular session or reads article/books that are related to the therapy.

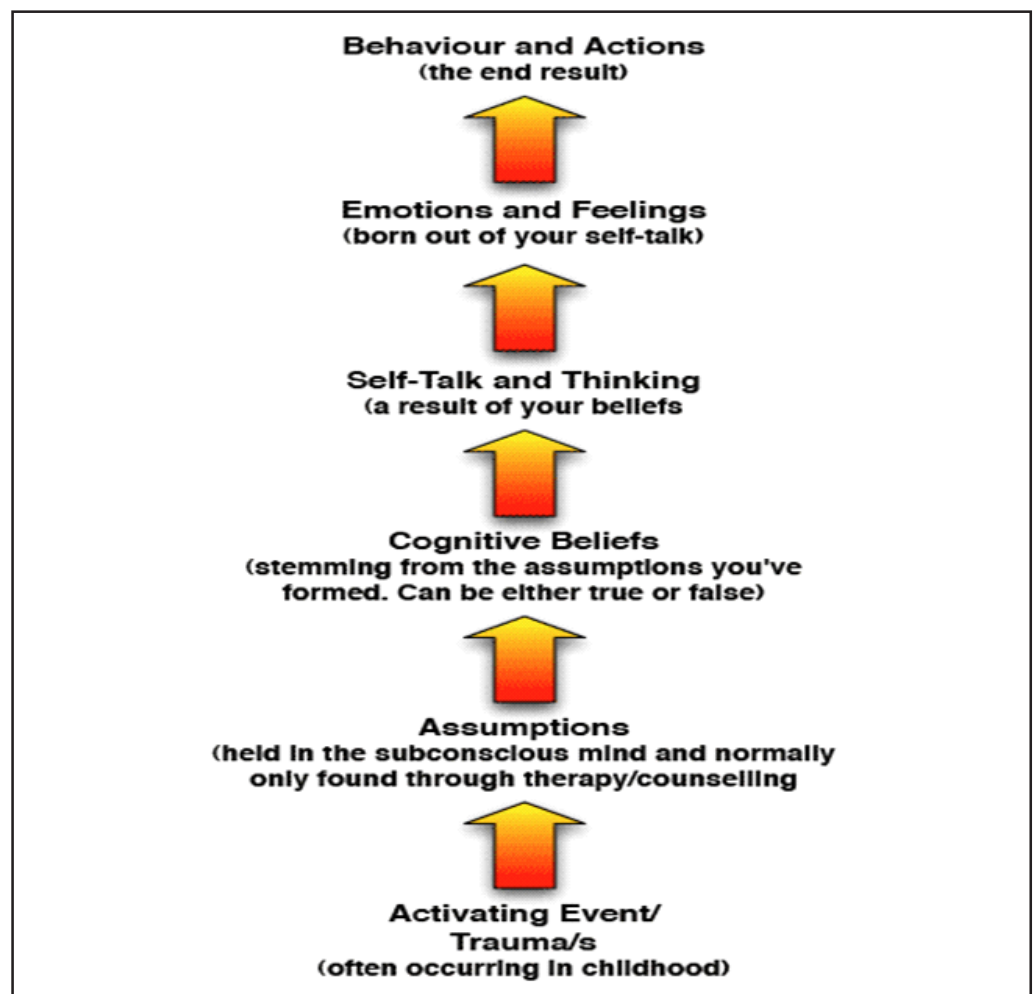
2.3.7 Hierarchical Structure of A-B-C

People experience events in their life. These events lead to assumptions (right or wrong) being made about that event. Once the assumption has been made, it gets placed into our subconscious mind. This assumption then generates beliefs or conclusions. These beliefs can either be true/accurate or false/inaccurate.

From these beliefs comes our thinking and self talk. From our thinking and self talk comes our feelings and emotions. Finally, our feelings and emotions give rise

to our behaviour and actions. The following diagram displays the way in which one wrong assumption can give birth to many beliefs, which in turn gives birth to many more self talk, emotional and behavioural issues.

Looking at this hierarchy structure, true change, that is life long and effective, can only happen if the core assumptions are uncovered and dealt with. The assumption is the base level which determines everything else in the hierarchy. If change was attempted by only addressing the behavioural level, sooner or later the emotions and self talk would cause that change to be negated. Will power can not override the internal feelings below the behavioural level. If change was attempted at the emotions and feelings level, then changed behaviour will follow, but at some point internal self talk, beliefs and assumptions will be too much to bear and cause the emotions then actions to revert back.



Thus Cognitive therapy involves getting down to the beliefs level.

Down at this level, lasting change can begin to occur.

Change in the belief system results in change in all the levels above it.

The only problem is that the underlying assumption that caused the false belief and then conflict is still lurking.

It means that if the assumption itself is incorrect/wrong/false, then it will generate other false beliefs.

What can occur in therapy then is that false belief upon false belief are traced, faced and replaced, but there always seems to be more.

The ONLY way to achieve true freedom from the shackles is to uncover the core assumption at the base level.

Once you find this and confront it, the false beliefs take care of themselves and no new ones are created, your self talk changes, your emotions and feelings turn positive and your actions follow suit.

2.4 LET US SUM UP

Behaviour therapy has been used successfully for a variety of disorders and can be easily taught. It requires less time than other therapies and is less expensive to administer. Although useful for circumscribed behavioural symptoms, the method cannot be used to treat global areas of dysfunction (e.g., neurotic conflicts, personality disorders).

Controversy continues between behaviourists and psychoanalysts, which is epitomised by Eysenck's statement: "Learning theory regards neurotic symptoms as simply learned habits; there is no neurosis underlying the symptoms, but merely the symptom itself. Get rid of the symptom and you have eliminated the neurosis."

Analytically-oriented theorists have criticized behaviour therapy by noting that simple symptom removal can lead to symptom substitution: When symptoms are not viewed as consequences of inner conflicts and the core cause of the symptoms is not addressed or altered, the result is the production of new symptoms. Whether this occurs remains open to question, however.

Whereas CBT is based on the concept that mental disorders are associated with characteristic alterations in cognitive and behavioural functioning and that this pathology can be modified with pragmatic problem-focused techniques. CBT is well established as a treatment for depression, anxiety disorders, and eating disorders. There is growing evidence that it can play an effective role in the clinical management of a large range of other disorders, including schizophrenia, bipolar disorder, and axis II conditions.

2.5 UNIT END QUESTIONS

- 1) What is Behaviour Therapy? Give a brief outline of its history.
- 2) What is modeling, and how it can be used in treating psychological problems?
- 3) Write a note on Exposure as an extinction approach.
- 4) Construct a hierarchy of fear arousing situation of any animal.
- 5) Define CBT and discuss the levels of cognitions.
- 6) According to CBT, What is the primary cause of mental disorders?
- 7) Explain the hierarchical structure of A-B-C model of CBT.

2.6 SUGGESTED READINGS

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